

1. Administrative & Study Identification

Full Study Title: Ravulizumab treatment outcomes in patients with Generalized Myasthenia Gravis (gMG) naive to complement inhibitors. Prospective, multicenter, non-interventional study (MG-ARCADIA) **Official Title:** Ravulizumab Treatment Outcomes in Patients With Generalized Myasthenia Gravis (gMG) Naive to Complement Inhibitors. Prospective, Multicenter, Non-interventional Study (MG-ARCADIA). **Short Title/Acronym:** MG-ARCADIA **Protocol Number:** D9281R00004 **NCT Number:** NCT06909253 **Posting ID:** 518152328825446770 **Trial Status:** RECRUITING **Sponsor/Company Name:** AstraZeneca **Investigational Product:** Ravulizumab (Ultomiris) | **ATC Code:** L04AA43 **Phase of Development:** Observational / Non-interventional (Not Applicable) **Medical Monitor:** AstraZeneca Clinical Operations Lead

2. Study Synopsis & Rationale

2.1 Study Objective Primary Objective: To collect longitudinal data on the management and clinical outcomes of patients with generalized Myasthenia Gravis (gMG) who are naive to complement inhibitors and receiving ravulizumab in the scope of routine clinical practice within the Polish National Drug Program (NDP). **Secondary Objectives:** To evaluate the real-world safety profile of ravulizumab and determine if patients can safely reduce their oral steroid use without worsening their gMG symptoms.

2.2 Background & Rationale Ravulizumab targets the complement system (inhibiting the C5 protein) and has demonstrated a good safety and efficacy record for gMG. This Polish, prospective, multicenter, observational cohort study aims to assess updated real-world effectiveness and safety data for complement protein C5 inhibitor therapy (C5IT) among patients with gMG in routine clinical practice. Specifically, researchers want to see if ravulizumab allows for the safe reduction of oral corticosteroids, offering patients a better long-term management strategy.

3. Study Design & Methodology

3.1 Design Framework Study Type: Observational, Cohort Study (Non-interventional) **Control Type:** Single-arm **Blinding:** Open-label **Randomization:** N/A

3.2 Planned Enrollment & Duration Target \$\$\$: 50 patients **Number of Sites:** Multicenter (Poland) **Estimated Study Start:** 15-May-2025 **Estimated Primary Completion:** 31-Dec-2029

4. Subject Selection (Eligibility Criteria)

4.1 Inclusion Criteria 1. Adult (aged ≥ 18 years) patients with gMG, naive to complement inhibitors, receiving ravulizumab treatment within the frames of the NDP in Poland. 2. Patients willing to participate in the study and have signed the Informed Consent Form (ICF). 3. Vaccination against *N. meningitidis*.

4.2 Exclusion Criteria 1. Those who plan to participate in a gMG clinical trial on or after the date of the first ravulizumab infusion through the NDP. 2. Cognitive incapacity, unwillingness, or language barriers precluding adequate understanding or cooperation.

5. Intervention & Treatment Plan

5.1 Investigational Regimen Dosage/Frequency: Administered according to routine clinical practice under the Polish NDP. **Route of Administration:** Intravenous (IV) infusion (concentrate for solution for infusion). **Duration of Treatment:** Up to 42 months of observation per patient (approx. 6 months baseline collection and 36 months of prospective follow-up). **Product Information (EMA):** [Ultomiris EPAR Product Information](#)

5.2 Concomitant & Prohibited Therapy Permitted: Routine care therapies for gMG, including oral corticosteroids (with a structured taper if clinically indicated) and acetylcholinesterase inhibitors. **Prohibited:** Concurrent participation in other interventional clinical trials; prior use of complement inhibitors.

6. Study Timeline & Visit Schedule

Visit ID	Window	Key Activities/Procedures	:---	:---
:---	Screening	Prior to Day 0	Informed Consent,	
			verification of <i>N. meningitidis</i> vaccination,	
			collection of	
			baseline (~6 months)	
			clinical history.	Baseline
				Day 0
				Initial ravulizumab administration through the NDP,
				gMG symptom
				baseline assessment.
		Interim	Over ~36 months	Routine
				clinical visits: assessment of daily activities,
				management of
				steroid tapering, and safety monitoring.
		End of Study	Month	
			~36	Final clinical status assessment, evaluation of long-term
				outcomes.

7. Outcome Measures (Endpoints)

7.1 Primary Outcome 1. Change from baseline in Myasthenia Gravis-Activities of Daily Living (MG-ADL) total score.

(Capturing longitudinal clinical outcomes and disease management metrics in patients receiving ravulizumab over the 36-month follow-up period).

7.2 Secondary Outcomes 1. Change from baseline in Myasthenia Gravis-Activities of Daily Living (MG-ADL) total score. 2. Change from baseline in MG-ADL subcomponent scores. 3. Change from baseline in Myasthenia Gravis (MG) composite scale. 4. Safety and tolerability assessments, specifically tracking mild to serious adverse events. 5. Changes in oral corticosteroid dosage to evaluate safe reduction without symptom exacerbation.

8. Safety & Adverse Event Reporting

Adverse Event (AE) Monitoring: Patient monitoring for AEs per routine clinical practice, with specific tracking of responses to steroid reduction and general tolerability. **Serious Adverse Events (SAEs):** Routine reporting within standard pharmacovigilance timelines. Special focus on monitoring for potential *N. meningitidis* infections due to C5 inhibition. **Data Monitoring Committee (DMC):** Overseen by the Sponsor's internal safety and observational data teams.

9. Statistical Analysis Plan (SAP) Summary

Analysis Populations: Observational Cohort Set (all enrolled patients receiving at least one dose of ravulizumab who provide consent). **Sample Size Justification:** $N = 50$, based on the expected number of eligible patients treated under the Polish NDP scope for gMG. **Handling of Missing Data:** Handled using standard methodologies for non-interventional real-world evidence (RWE) studies, utilizing available longitudinal data.

10. Quality Assurance & Regulatory Compliance

GCP Compliance: This study will be conducted in accordance with Good Pharmacoepidemiology Practices (GPP) and the Declaration of Helsinki. **Monitoring Plan:** Data abstraction from routine clinical records into electronic Case Report Forms (eCRFs). **Audit Trail:** Standard digital audit trailing for eCRFs and patient data privacy protection according to local Polish regulatory guidelines.

11. Study Identifiers & Governance

Primary ID: D9281R00004 **Registry Number:** NCT06909253 **Posting ID:** 518152328825446770 **Sponsor/Lead Organization:** AstraZeneca **Study Title:** MG-ARCADIA **Official Regulatory Title:** Ravulizumab treatment outcomes in patients with Generalized Myasthenia Gravis (gMG) naive to complement inhibitors. Prospective, multicenter, non-interventional study (MG-ARCADIA).

12. Clinical Framework (gMG Specific)

Primary Condition: Generalized Myasthenia Gravis (gMG) **Disease Area Preferred Name:** Myasthenia Gravis, Generalized **Standard Clinical Classifications:** MeSH Code: D009157 | SNOMED ID: 722510008 **Disease Definition:** Generalized Myasthenia Gravis is a chronic autoimmune neuromuscular disorder characterized by fluctuating muscle weakness and fatigability, which worsens with activity and improves with rest. It occurs when the immune system mistakenly attacks and damages acetylcholine receptors or other proteins at the neuromuscular junction, preventing the proper transmission of nerve impulses to the muscles. This leads to impaired muscle contraction affecting various voluntary muscles, including those controlling eye movements, facial expression, swallowing, speech, and limb movement. **Diagnosis Threshold:** Confirmed clinical diagnosis of gMG, naive to complement inhibitors **Medical Methodology:** Complement C5 inhibitor therapy **Optimization Target:** Improvement of neuromuscular symptoms and safe reduction of oral steroids

13. Study Procedures & Methodology

Management Pathway: Routine clinical pathway within the Polish National Drug Program (NDP) **Education Protocol (HCP):** NDP prescribing compliance and eCRF data tracking **Education Protocol (Patient):** Counseling on the necessity of *N. meningitidis* vaccination prior to treatment and reporting of infection symptoms **Quality Audit Mechanism:** Real-world data verification from site medical records

14. Observation & Follow-Up Schedule

Total Duration: ~42 months (~6 months baseline collection and ~36 months prospective follow-up) **Onsite Visit Cadence:** Aligned with routine clinical care and NDP infusion schedules **Remote Monitoring Frequency:** As indicated by routine clinical practice **Checklist Review Interval:** Routine follow-up intervals over the 36-month observation period

15. Sign-off & Approval

Role	Name	Signature	Date		:---	:---	:---	:---
Principal Investigator	[Name]	_____	[DD-MMM-YYYY]					
Clinical Operations Lead	[Name]	_____	[DD-MMM-YYYY]					Lead
Statistician	[Name]	_____	[DD-MMM-YYYY]					